NCCP 00507 · VERSION 6

Weekly paclitaxel + pertuzumab + trastuzumab

Paclitaxel on Days 1, 8 and 15 of each 21-day block, for up to eight cycles; IV pertuzumab and trastuzumab on Day 1. This NCCP alternative applies when docetaxel is unsuitable, not tolerated or caused significant toxicity. Antibodies may continue after the taxane stops while beneficial and tolerated.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Heart

Trastuzumab / pertuzumab

Keep echo appointments; report breathlessness or swelling.

3 Blood / infection

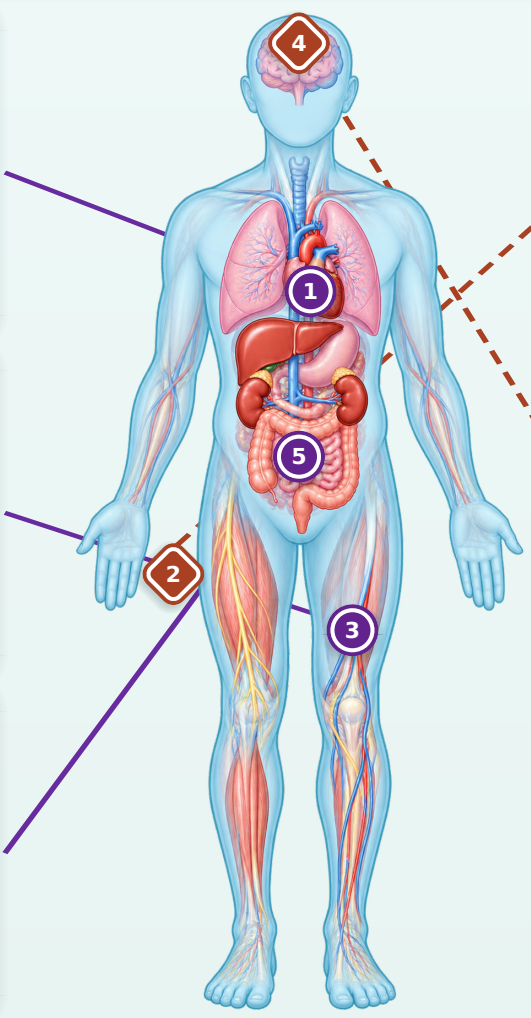
Chemotherapy phase

Fever, shivers or suddenly unwell: call immediately.

5 Bowel

Pertuzumab / chemotherapy

Report diarrhoea promptly, especially with poor intake.



2 Hands / feet

Paclitaxel

Report numbness or tingling before the next dose.

4 Hair / scalp

Chemotherapy phase

Hair loss is common. Ask about scalp cooling before treatment where suitable.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

The map shows selected effects of the medicines named here. Report persistent effects from earlier treatment too.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

Your treatment, at home.

Important medicine-specific effects

Paclitaxel

- Low blood counts and infection
- Numbness / tingling, sometimes lasting
- Hair loss, muscle or joint pain
- Infusion reaction
- Mouth, bowel, skin and nail effects

Pertuzumab

- Diarrhoea and dehydration
- Infusion / allergic reaction
- Heart pumping-function reduction
- Rash and fatigue

Trastuzumab

- Heart pumping-function reduction / heart failure
- Infusion or allergic reaction
- Diarrhoea and fatigue
- Rare serious lung effects

Surgery, radiation and monitoring: open the whole-treatment map or scan the QR.

Make the plan practical

The correct supportive prescription

Follow the script for the active medicine, route and treatment day. Do not carry chemotherapy anti-sickness medicines automatically into an antibody-only phase.

Tests before treatment

FBC, kidney and liver profile at baseline and before every 21-day cycle. The team reviews the necessary results before a planned dose. Confirm the blood-count check needed before each additional vinorelbine or paclitaxel dose; a Day 1 result does not automatically authorise a later dose.

Keep heart checks on the plan

An echo every 12 weeks is separate from blood tests or cancer imaging. Ask who books and reviews it.

Plan for symptoms

Keep your agreed contact numbers and instructions. New symptoms can need assessment before a routine appointment.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00507/

Sources and review status are available online. Follow your own prescription and hospital instructions.